

Class 4 – Humerus Fracture: Patient Care Guide

A patient-friendly guide for people with a humerus fracture — a break in the upper arm bone that runs from the shoulder to the elbow. It covers causes, symptoms, diagnosis, treatment, healing, exercises, warning signs, and common questions.

Medical disclaimer: This document is an educational reference created for a patient-support knowledge base and for use in a retrieval-augmented (RAG) AI assistant. It is NOT a substitute for professional medical advice, diagnosis, or treatment. Healing times, medicines, and exercises differ for every person. Always follow the instructions of your own doctor, orthopaedic surgeon, or physiotherapist. In an emergency, contact local emergency services immediately.

1. Introduction

A humerus fracture is a break in the humerus, the long bone of the upper arm that connects the shoulder to the elbow. It is also called a broken upper arm. The break can be near the shoulder (proximal), in the middle (shaft), or near the elbow (distal).

Which bone is involved: the humerus. A proximal humerus fracture is near the shoulder joint, a humeral shaft fracture is in the middle, and a distal humerus fracture is near the elbow.

Common causes: falls onto the arm or shoulder, road accidents, sports injuries, and direct blows. In older adults, even a minor fall can break a weakened humerus. How common it is: humerus fractures, especially proximal ones, are common, particularly in older adults with osteoporosis.

Who is at higher risk: older adults with weak bones, people who fall, athletes in contact and wheel sports, and those in high-impact accidents. Severity ranges from an undisplaced crack (mild) to a shattered, displaced, or open fracture (severe).

Types of Fractures Affecting This Bone

A humerus fracture can happen in several patterns. Common fracture types include:

- Hairline fracture (a very thin, small crack in the bone).
- Stress fracture (a tiny crack caused by repeated force or overuse).
- Greenstick fracture (the bone bends and cracks on one side; common in children).
- Closed fracture (the bone breaks but the skin stays intact).
- Open (compound) fracture (the broken bone breaks through the skin).
- Comminuted fracture (the bone breaks into three or more pieces).
- Spiral fracture (a break that twists around the bone, often from a twisting injury).
- Impacted fracture (the broken ends are driven into each other).
- Transverse fracture (a straight break across the bone).
- Oblique fracture (a slanted or angled break across the bone).

Proximal humerus fractures (near the shoulder) are the most common, especially in older adults, while shaft fractures can injure the radial nerve that wraps around the bone.

2. Anatomy

The humerus is surrounded by powerful muscles, important nerves, and major vessels.

- Bone involved: the humerus, forming the ball of the shoulder joint at the top and part of the elbow at the bottom.
- Nearby muscles: the deltoid and rotator cuff move the shoulder; the biceps and triceps bend and straighten the elbow; these pull on the bone during healing.
- Tendons: the rotator cuff tendons attach near the top of the humerus and are vital for shoulder movement.
- Ligaments: support the shoulder and elbow joints at each end of the bone.
- Blood vessels: the brachial artery runs along the inner arm to supply the forearm and hand; injury can affect blood flow.
- Nerves: the radial nerve wraps closely around the middle of the humerus and can be injured in shaft fractures, causing 'wrist drop' (difficulty lifting the wrist). The axillary nerve near the shoulder can be affected in proximal fractures.

Because the radial nerve lies against the humerus, a shaft fracture can stretch or pinch it and weaken wrist and finger lifting; this often recovers but must be checked. Strong muscles pulling on the bone also influence how the fracture is treated.

3. Common Causes

Most fractures happen when a force is stronger than the bone can handle. The main causes are:

- Falls: slipping, tripping, or falling onto an outstretched hand or arm — the most common cause overall.
- Road traffic accidents: high-speed impacts from car, motorbike, or cycle crashes.
- Sports injuries: contact sports, skating, cycling, gymnastics, and falls during play.
- Workplace accidents: falls from height, machinery injuries, or heavy objects.
- Direct trauma: a hard blow or hit directly on the bone.
- Twisting injuries: the limb turns while the hand or body stays fixed.
- Heavy object injuries: something heavy falling onto the limb.
- Osteoporosis: weak, thin bones that break easily, especially in older adults.
- Bone tumors: growths that weaken the bone (pathological fracture).
- Repetitive stress: repeated overuse leading to stress fractures over time.

For the humerus specifically, a fall onto the shoulder or arm is the leading cause in older adults, while high-energy accidents cause shaft fractures in younger people.

4. Symptoms

Common signs and symptoms of a humerus fracture include:

- Pain in the upper arm, worse with movement.
- Swelling and bruising over the arm and shoulder.
- A deformed or shortened upper arm.

- Inability to lift or move the arm.
- A grinding or cracking feeling with movement.
- Wrist drop or weakness lifting the wrist/fingers (possible radial nerve injury).
- Numbness or tingling in the arm or hand.
- Bruising that spreads down toward the elbow over days.
- In an open fracture, bone may protrude through the skin.

Emergency warning signs: a cold, pale, or blue hand, no pulse at the wrist, inability to lift the wrist or fingers, bone through the skin, or severe uncontrolled pain. Seek emergency care immediately.

5. Diagnosis

Doctors confirm a fracture using a physical check and imaging tests. Each test gives different information.

Physical Examination and Medical History

The doctor asks how the injury happened and checks for pain, swelling, bruising, deformity, tenderness, movement, pulse, skin colour, and sensation. Telling the doctor about past fractures, osteoporosis, medicines, and other illnesses helps plan the right treatment.

Imaging Tests

- X-ray: the first and main test. It shows most fractures, the exact location, and whether the bone has moved out of place (displacement).
- CT scan: gives detailed 3-D images. Used for complex breaks, joint involvement, or when the X-ray is unclear, to plan surgery.
- MRI: shows soft tissues, bone bruising, and hairline or stress fractures that may not appear on an X-ray. Also checks nearby ligaments, tendons, and nerves.
- Ultrasound: sometimes used, especially in children, to look at bone surfaces and soft-tissue swelling without radiation.
- Bone scan: can detect very small stress fractures or hidden fractures that other tests miss, by showing areas of increased bone activity.

For a humerus fracture, X-rays of the whole upper arm including the shoulder and elbow are standard. A CT scan is often used for proximal (shoulder) fractures to see the fragments before surgery. The doctor will specifically test the radial nerve (wrist and finger lifting) in shaft fractures.

6. Treatment Options

Treatment depends on the type of break, how far the bone has moved, and your age and health. It falls into non-surgical and surgical options.

Non-Surgical Treatment

Many humerus fractures, especially shaft and undisplaced proximal fractures, heal well without surgery because the surrounding muscles help hold the bone.

- Functional brace: a special removable brace (often after a first splint) supports a shaft fracture while allowing the muscles to align the bone.

- Sling or shoulder immobiliser: rests the arm, especially for proximal fractures.
- Cast or splint: a U-slab or splint may be used early, particularly with swelling.
- Ice therapy: reduces pain and swelling in the first days.
- Elevation and support: keeping the arm supported reduces discomfort.
- Pain medications: paracetamol for pain relief.
- Anti-inflammatory medicines: such as ibuprofen, if approved by your doctor.

When recommended: non-surgical care suits stable fractures where the bone is in a good position. Advantages: no surgery, lower infection risk, and no anaesthesia. Limitations: needs a well-set position, the cast can cause stiffness, and it may not hold badly displaced or unstable fractures.

Surgical Treatment

Surgery is used when the bone is badly out of place, broken into many pieces, involves a joint, is an open (compound) fracture, or fails to heal with a cast. The aim is to put the pieces back in the correct position and hold them still while they heal.

- ORIF (Open Reduction Internal Fixation): the surgeon opens the skin, sets the bone in the right position (reduction), and fixes it with metal implants.
- Plates: flat metal supports screwed along the bone to hold the pieces together.
- Screws: hold fragments together, alone or with a plate.
- Pins (K-wires): thin wires that hold small pieces, often used in fingers and in children; usually removed later.
- Rods (intramedullary nails): metal rods placed inside the hollow centre of a long bone to support it.
- External fixator: a frame outside the body connected to the bone by pins; used for severe, open, or badly contaminated fractures.
- Bone graft: extra bone (from your own body or a donor) added to help healing when there is bone loss or slow healing.

Indications (when surgery is needed): displaced or badly angled fractures, open fractures, fractures with a nerve or blood vessel injury, fractures that will not stay aligned in a brace, and many displaced proximal fractures in active patients. Shaft fractures are often fixed with a plate or an intramedullary rod; proximal fractures may need plates, screws, or a shoulder replacement in severe cases.

Recovery Expectations and Risks After Surgery

- Recovery: a protective cast, splint, or sling is often used after surgery, followed by physiotherapy. Implants usually stay in place unless they cause problems.
- Risks: infection, bleeding, nerve or blood vessel injury, implant irritation or loosening, stiffness, blood clots, and reaction to anaesthesia.
- Most people regain good function, but full recovery takes weeks to months and depends on the injury and following rehabilitation.

7. Healing Timeline

Healing time varies by age, general health, fracture type, and how well instructions are followed. The table gives typical ranges.

Group	Typical healing / recovery
Children	Often 4–6 weeks to unite; children's humerus heals and remodels well.
Adults	About 8–12 weeks for bone union; shaft fractures may take longer.
Older adults	12 weeks or more; healing slower with osteoporosis.
Athletes	Union in 8–12 weeks; return to sport often 4–6 months.

Stages of healing: initial healing (inflammation and soft callus) in the first weeks, bone union (hard callus bridges the break), and functional recovery (regaining strength and movement). Regaining full overhead shoulder movement is often the slowest goal, especially after proximal fractures.

- Return to work: desk work in 1–3 weeks; heavy manual/overhead work often 12 weeks or after clearance.
- Return to sports: non-contact around 10–12 weeks; contact/throwing sports usually 4–6 months.
- Full recovery: 4–6 months for most; shoulder stiffness can take longer to fully resolve.

Factors that delay healing: smoking, alcohol, poor nutrition, diabetes, osteoporosis, infection, poor blood supply, older age, certain medicines (such as steroids), and not resting the injured limb.

8. Recovery Stages

Knowing what to expect at each stage helps you feel reassured. Your own timeline may differ slightly.

Stage	What usually happens
Week 1	Pain and swelling peak; arm rested in sling/brace; move fingers and wrist gently.
Week 2	Pain easing; keep hand, wrist, and (if allowed) gentle pendulum shoulder movement.
Week 3–4	Bone knitting; brace continues; gentle guided shoulder/elbow movement may start.
Week 5–8	Bone uniting; more active movement; physiotherapy progresses.
Month 3	Bone usually united; focus on shoulder range and arm strength.
Month 6	Most movement and strength restored; heavier activity resumed.

9. Daily Care Instructions

- Keep any cast or brace dry: cover it while bathing to protect the skin and material.
- Sleeping position: many people sleep more comfortably semi-upright (propped on pillows or in a recliner) for the first weeks; keep the arm supported.
- Bathing: use a waterproof cover or sponge-bathe; keep the arm supported.
- Clothing: wear loose, front-opening clothes; dress the injured arm first.
- Walking: normal walking is fine; keep the arm in the sling for support.
- Driving: avoid driving until the arm moves safely and you are cleared — often several weeks.
- Returning to office: light desk work is often possible within 1–3 weeks.

- Returning to school: children can attend once comfortable, avoiding sport.
- Household activities: avoid lifting, reaching overhead, and carrying with the injured arm.

10. Pain Management

Some pain is normal after a fracture, especially in the first days. These safe, simple methods help control it.

- Ice packs: wrap ice in a cloth and apply for 15–20 minutes several times a day to reduce pain and swelling (never put ice directly on the skin).
- Elevation: keep the injured limb raised above the level of the heart to reduce throbbing and swelling.
- Rest: avoid using the injured limb for gripping, lifting, or bearing weight.
- Painkillers: paracetamol (acetaminophen) is commonly used. Anti-inflammatory medicines such as ibuprofen may be advised. Take only the dose your doctor approves.
- Safe positioning: support the limb on a pillow and avoid letting it hang down.
- Relaxation techniques: slow breathing, distraction, and gentle music can reduce the sensation of pain.

When pain is abnormal: pain that suddenly becomes severe, is not relieved by medicine, comes with numbness, a cold or blue hand, or a very tight cast may signal a serious problem such as compartment syndrome or poor blood supply. Seek medical help urgently.

11. Diet for Bone Healing

Good nutrition speeds up bone healing. The nutrients below all play a role. Try to eat a balanced diet that includes them every day, and follow any special diet advice from your doctor or dietitian.

Key Nutrients for Bone Healing

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Calcium	Main mineral that builds and repairs bone. About 1000–1200 mg/day for adults.	Milk, curd, paneer, cheese, ragi, tofu, green leafy vegetables, sesame seeds	Small fish eaten with bones (sardines), dairy
Vitamin D	Helps the body absorb calcium. About 600–800 IU/day (more if deficient).	Sunlight, fortified milk and cereals, mushrooms	Egg yolk, fatty fish (salmon, mackerel), fish liver oil
Protein	Builds the collagen framework of bone. About 1.0–1.5 g per kg body weight.	Dal, beans, chickpeas, rajma, soya, paneer, tofu, nuts	Eggs, chicken, fish, lean meat
Vitamin C	Needed to make collagen for bone repair. About 65–90 mg/day.	Oranges, lemon, amla, guava, capsicum, tomato, broccoli	(Mainly from plant foods)
Vitamin K	Helps bind calcium into bone. About 90–120 mcg/day.	Spinach, kale, broccoli, cabbage, green leafy vegetables	Eggs, some dairy

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Magnesium	Supports bone structure and vitamin D use. About 310–420 mg/day.	Nuts, seeds, whole grains, bananas, dark leafy greens	Fish
Phosphorus	Works with calcium to harden bone. About 700 mg/day.	Dairy, beans, lentils, whole grains, nuts	Meat, fish, poultry, eggs
Zinc	Helps bone cells grow and repair. About 8–11 mg/day.	Whole grains, legumes, seeds, nuts	Meat, shellfish, eggs
Omega-3 fats	Reduce inflammation and support healing.	Flaxseed, chia seeds, walnuts, soybean oil	Fatty fish (salmon, mackerel, sardines)
Collagen	Forms the flexible base of bone. Supported by protein + vitamin C.	Body makes it from protein-rich plant foods + vitamin C	Bone broth, chicken, fish, eggs

Drink enough water and, if your doctor recommends, take calcium and vitamin D supplements. Do not take high-dose supplements on your own without medical advice.

12. Foods to Avoid

Some foods and habits slow bone healing by reducing calcium absorption, increasing inflammation, or harming blood supply. Limit or avoid the following while your fracture heals.

- Alcohol: slows new bone formation and reduces calcium and vitamin D use. Best avoided completely during healing.
- Smoking and tobacco: reduce blood flow to the bone and are a leading cause of delayed healing and nonunion. Stopping greatly improves healing.
- Sugary foods: too much sugar can increase inflammation and displace healthy foods.
- Soft drinks and colas: high phosphoric acid and sugar may reduce calcium balance.
- Excess caffeine: large amounts of coffee, tea, or energy drinks can reduce calcium absorption; keep intake moderate.
- Processed and packaged food: often high in salt and low in nutrients.
- High sodium (salt) intake: makes the body lose more calcium in urine.
- Junk and deep-fried food: high in unhealthy fat and low in bone-building nutrients.

In short, a fresh, home-cooked, balanced diet with less salt, sugar, alcohol, and no smoking gives your bone the best chance to heal quickly.

13. Physical Therapy and Rehabilitation Exercises

Physiotherapy restores movement, strength, and function. Start only when your doctor allows, and stop any exercise that causes sharp or increasing pain. Below are typical stages — your therapist will tailor them to you.

Early Stage

- Finger, wrist, and elbow movements — purpose: prevent stiffness while the shoulder rests. Repetitions: gentle, 10 each, several times daily.
- Pendulum exercises (letting the arm hang and gently swinging in small circles) — purpose: gentle early shoulder motion. Precaution: only if your doctor allows.

Middle Stage

- Assisted shoulder movements (using the good arm or a stick) — purpose: regain range without straining. Repetitions: 10 slow reps. Precaution: stay within a pain-free range.
- Active elbow bending and straightening — purpose: reduce elbow stiffness.
- Gradual shoulder raises to the front and side — purpose: restore movement.

Advanced Stage

- Resistance-band shoulder and rotator-cuff exercises — purpose: rebuild strength. Repetitions: 10–15, 2–3 sets. Precaution: start light.
- Overhead reaching and strengthening — purpose: restore full function.
- Sport-specific and lifting progression — purpose: safe return. Precaution: increase load slowly; stop if pain returns.

General exercise precautions: move slowly, never push through sharp pain, stop if you feel numbness or dizziness, and follow the exact repetitions your therapist advises. When to stop: stop and call your therapist if pain worsens, swelling increases, or movement suddenly decreases.

14. Activities to Avoid During Healing

Avoiding certain activities protects the healing bone and implants.

Activity	How long to avoid (typical)
Lifting or carrying with the injured arm	Until healed — often 10–12 weeks
Reaching or working overhead	About 3 months or until cleared
Push-ups, pull-ups, weight training	About 3–4 months
Contact and throwing sports	About 4–6 months
Driving with a braced/painful arm	Until safe control and clearance
Carrying heavy bags on that side	Until full strength returns (about 3 months)

15. Possible Complications

Most fractures heal well, but knowing the possible problems helps you spot them early. Report any of these to your doctor.

- Delayed union: the bone heals more slowly than expected. Prevented by good nutrition, not smoking, and following restrictions.
- Nonunion: the bone fails to join. Signs are ongoing pain and movement at the break site; may need surgery or a bone graft.

- Malunion: the bone heals in the wrong position, causing deformity or reduced function; sometimes needs correction.
- Infection: mainly with open fractures or after surgery. Signs are fever, redness, warmth, swelling, pus, or bad smell.
- Stiffness: nearby joints become stiff from being kept still; prevented by physiotherapy and gentle movement as advised.
- Nerve injury: causes numbness, tingling, or weakness; often improves but should be checked.
- Blood vessel injury: rare but serious; causes a cold, pale, or blue limb and needs emergency care.
- Chronic (long-lasting) pain: pain that continues after healing; managed with therapy and pain specialists.
- Arthritis: joint fractures can lead to wear-and-tear arthritis years later.
- Reduced mobility: lasting loss of full movement, reduced by good rehabilitation.
- Complex Regional Pain Syndrome (CRPS): an uncommon condition with severe pain, swelling, skin colour and temperature changes; early treatment gives the best result.

16. Red Flag Symptoms — Go to Hospital Immediately

Get emergency medical help right away if you notice any of the following. These can mean the blood supply, nerves, or the healing is in danger.

- Severe or rapidly increasing swelling.
- Fingers (or toes) turning blue, grey, or very pale.
- The hand or limb feels cold to touch.
- Numbness or loss of feeling that does not go away.
- Fever or chills (a sign of infection).
- Bleeding that will not stop.
- Severe pain that is not relieved by rest, elevation, or prescribed medicine.
- The cast becomes too tight or the limb feels squeezed.
- The cast breaks, cracks, or becomes loose.
- Pus, discharge, or fluid coming from under the cast or a wound.
- A bad smell coming from the cast or wound.
- Bone is visible through the skin (open fracture).
- Difficulty breathing, chest pain, or coughing after the injury (a possible blood clot or fat embolism — a rare but serious emergency).

Compartment syndrome warning: severe deep pain, tightness, numbness, and pain when the fingers are gently stretched can signal dangerous pressure build-up. This is an emergency — do not wait.

17. Follow-Up Schedule

Regular follow-up lets the doctor check that the bone is healing in the right position. A typical schedule is below.

Visit	What the doctor usually checks
Week 1	Check swelling, brace/sling fit, blood flow, and radial nerve (wrist lift); X-ray.
Week 2	Confirm alignment held; check the wound if operated.
Week 4	X-ray for early healing; begin/progress gentle movement.
Week 6	Check union; advance physiotherapy for shoulder and elbow.
Week 12	Confirm union; assess range, strength, nerve recovery, and plan return to activity.

18. Returning to Normal Life

These are general guides for when people usually return to activities. Always get your doctor's clearance first.

Activity	Typical time to return
School	A few days to 1–2 weeks (no sport)
Office (desk job)	1–3 weeks
Driving	After safe arm control and clearance — often 6–10 weeks
Gym (upper body)	About 3–4 months, building up gradually
Swimming	After brace off and skin/wound healed — about 12 weeks
Cycling	Light, stable cycling around 10–12 weeks (avoid falls)
Running	Often 6–8 weeks once the arm is stable
Cricket / Football	About 4–6 months with clearance
Weight lifting	About 4–6 months, increasing load slowly

19. Prevention of Future Fractures

After one fracture, it is worth protecting your bones for the future. These steps lower the chance of breaking a bone again.

- Bone-strengthening exercises: regular resistance and muscle-strengthening activity keeps bones dense.
- Weight-bearing exercise: walking, jogging, dancing, and stair climbing build bone strength.
- Balance training: yoga, tai chi, and balance drills reduce falls, especially in older adults.
- Vitamin D: from sunlight, diet, or supplements as advised, to help absorb calcium.
- Calcium intake: eat enough calcium-rich foods every day.
- Protective equipment: wrist guards, elbow pads, and helmets during sports and skating.
- Safe workplace habits: follow safety rules, use guards, and prevent falls from height.
- Sports safety: warm up, use correct technique, and wear the right gear.
- Fall prevention: good footwear, adequate lighting, and vision checks.

- Home safety tips: remove loose rugs, add grab bars, keep floors dry and clutter-free.
- Osteoporosis screening: a bone density (DEXA) scan for older adults and high-risk people, with treatment if bones are weak.

20. Frequently Asked Questions (FAQ)

Q1. Can I move my fingers?

Yes. Move your fingers, wrist, and (when allowed) elbow gently while the shoulder rests. This prevents stiffness and swelling and keeps the hand working.

Q2. Can I sleep on the injured arm?

No. Many people sleep better semi-upright, propped on pillows, with the arm supported. Avoid lying on the injured side.

Q3. Can I remove the brace or sling?

Only as your doctor instructs. A functional brace usually stays on most of the time; some are removed briefly for washing and exercises.

Q4. When can I drive?

Usually after several weeks, once you can control the car safely and are cleared — often around 6–10 weeks.

Q5. Can I exercise?

Gentle hand, wrist, and lower-body exercise is fine early. Shoulder and upper-body strengthening comes later, on your doctor's advice.

Q6. Can I lift weights?

Not until the bone heals and you are cleared — usually about 4–6 months, starting light.

Q7. Is swelling and bruising normal?

Yes. Bruising can spread down the arm toward the elbow over days — this is normal. Report severe swelling or a cold, blue hand urgently.

Q8. Is itching inside the cast/brace normal?

Yes, mild itching is common. Do not insert objects to scratch; cool air can help.

Q9. Can I travel?

Short travel is fine once pain is controlled and the arm is supported. Ask before long journeys.

Q10. Can I fly?

Usually yes; tell your doctor, especially soon after injury, as swelling can increase.

Q11. Can I get the cast or brace wet?

No, keep it dry with a waterproof cover during bathing to protect the skin and material.

Q12. How long should I wear the sling or brace?

Often several weeks (commonly 4–8), depending on the fracture and your doctor's plan.

Q13. When should I see a doctor urgently?

For a cold or blue hand, no wrist pulse, inability to lift the wrist/fingers, severe pain, fever, or a wound with discharge.

Q14. My wrist won't lift — is that serious?

Difficulty lifting the wrist (wrist drop) can mean radial nerve irritation, common with shaft fractures. It often recovers but must be checked by your doctor.

Q15. Why is my shoulder so stiff?

After weeks in a sling the shoulder stiffens easily. This is common and improves with physiotherapy; regaining overhead movement takes time.

Q16. Will I need surgery?

Not always. Many shaft and undisplaced fractures heal in a brace. Displaced, open, or nerve-injury fractures often need a plate or rod.

Q17. Do the plates or rods need to be removed?

Usually they can stay for life and are removed only if they cause pain or problems.

Q18. Is some pain normal weeks later?

Mild aching during healing and physiotherapy is normal. Sharp or worsening pain should be reviewed.

Q19. What foods help my arm heal?

Calcium, vitamin D, protein, and vitamin C support bone repair. Avoid smoking and alcohol, which slow healing.

Q20. Can I go to work?

Desk work is often possible in 1–3 weeks. Heavy or overhead work usually needs about 12 weeks or clearance.

Q21. Will my arm be weak after the brace comes off?

Yes, temporary weakness and stiffness are expected. Physiotherapy rebuilds strength and movement.

Q22. Can I play sports again?

Yes, most people do. Non-contact around 10–12 weeks and contact/throwing sports around 4–6 months, after clearance.

Q23. Can I raise my arm overhead again?

Most people regain overhead movement with physiotherapy, though it is often the last motion to fully return.

Q24. Why do I sleep better sitting up?

Lying flat can increase arm pain and pressure. Sleeping semi-upright supports the arm and is more comfortable early on.

Q25. How can I prevent another humerus fracture?

Strengthen bones with exercise, get enough calcium and vitamin D, prevent falls at home, and get osteoporosis screening if at risk.

Q26. Can I use my hand for eating and phone use?

Yes, gentle use of the hand for light tasks like eating or texting is usually fine if it does not hurt the upper arm.

21. Metadata (for Retrieval)

Structured tags to help the AI assistant retrieve this document accurately.

Field	Value
Fracture Name	Humerus Fracture
Bones Involved	Humerus (upper arm bone)
Body Region	Upper limb (upper arm, shoulder to elbow)
Common Causes	Falls onto arm/shoulder, road accidents, sports, direct trauma, osteoporosis
Symptoms	Pain, swelling, deformity, unable to lift arm, wrist drop, numbness, bruising
Treatment Options	Functional brace, sling, splint; ORIF with plate or intramedullary rod, screws, shoulder replacement in severe proximal fractures
Healing Duration	Children 4–6 weeks; adults 8–12 weeks; full recovery 4–6 months
Rehabilitation Duration	10–20 weeks of physiotherapy; shoulder stiffness may take longer
Emergency Signs	Cold/blue hand, absent pulse, wrist drop, open bone, uncontrolled pain
Keywords	humerus fracture, broken upper arm, proximal humerus fracture, humeral shaft fracture, radial nerve injury, wrist drop
Synonyms	Broken upper arm, cracked humerus, upper arm bone fracture
Related Injuries	Shoulder fracture, elbow fracture, rotator cuff injury, radial nerve palsy